INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Everolimus + exemestane

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00322
Version 4

Continuous oral treatment

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Sources and further information	15

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Exemestane lowers oestrogen production while everolimus blocks an intracellular growth pathway that can contribute to endocrine resistance. The aim in this setting is to control cancer and delay progression.

MTOR-TARGETED TABLET

Everolimus

Inhibits mTORC1 growth signalling

Growth signals reach mTOR → Everolimus blocks mTORC1 → Cancer-cell growth signalling decreases

Everolimus binds the intracellular protein FKBP12; this complex inhibits mTORC1, affecting protein synthesis and growth signalling. This pathway can support endocrine resistance, which explains pairing it with an oestrogen-lowering medicine. The same pathway operates in normal tissues, contributing to mouth ulcers, infection risk and metabolic changes.

Report new mouth soreness, cough/breathlessness, fever/infection symptoms, diarrhoea or excessive thirst promptly. A new cough must not simply be labelled drug pneumonitis without checking other causes.

STEROIDAL AROMATASE-INHIBITOR TABLET

Exemestane

Irreversibly inhibits aromatase and lowers oestrogen production

Aromatase makes oestrogen → Exemestane blocks aromatase → Less oestrogen drives cancer cells

After menopause, aromatase in tissues converts androgen precursors into oestrogen. Exemestane is an irreversible aromatase inhibitor that reduces this supply to hormone-sensitive cancer cells. It does not directly switch off functioning ovaries; the menopausal indication must be confirmed.

Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning can be silent, so keep the agreed bone-health reviews.

Your treatment aim

Who this treatment is for

Postmenopausal HR-positive/HER2-negative advanced disease after recurrence/progression following an NSAI, without symptomatic visceral disease. The intended endocrine partner is exemestane; do not automatically substitute another endocrine drug.

NCCP 00322 v4 · official regimen

Schedule and duration

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date. Continue while beneficial and tolerated; review each medicine separately if everolimus is held.

NCCP 00322 v4 · official regimen

What the studies tell us

Adding everolimus to exemestane delayed cancer growth compared with exemestane alone in the study. The trial did not show a clear improvement in how long people lived overall. This older study cannot give a precise estimate for someone previously treated with modern CDK4/6 combinations.

How to interpret this

Study results describe groups with defined treatment and eligibility. Ask about your own likely benefits, important harms, alternatives and remaining uncertainty.

BOLERO-2 final PFS, Yardley et al 2013 · BOLERO-2 final OS, Piccart et al 2014

BOLERO-2

Studies describe groups. Ask how closely the treatment and study population match your situation.

Your team can discuss the strength and limitations of the evidence alongside treatment risks and alternatives.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology team promptly for new or worsening cough or breathlessness, fever or infection symptoms, mouth sores that prevent drinking/eating, severe diarrhoea or dehydration, or marked thirst and frequent urination. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00322 v4 · official regimen · EMA Afinitor SmPC; product information updated 20 November 2025, checked 20 October 2026

THE WHOLE TREATMENT JOURNEY

Everolimus + exemestane: treatment and checks

Your treatment pattern

Continuous oral treatment

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Postmenopausal HR-positive / HER2-negative advanced breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First eight weeks	Four-week reviews	Longer-term treatment	Hold / change / completion
Treatment NCCP 00322 · SMPC	Confirm both prescriptions	Both medicines daily; no rest week	Continue daily unless held	Review benefit and tolerability	Confirm each medicine's plan separately
Blood counts / renal / liver NCCP 00322 · SMPC	Baseline, including urea and urinary protein	Every four weeks or as indicated	Every four weeks or as indicated	Individualise additional renal/protein checks	Review abnormalities before restart
Glucose / lipids NCCP 00322 · SMPC	Glucose, cholesterol and triglycerides	Lipids every four weeks; glucose as indicated	Continue metabolic monitoring	Adapt to diabetes, results and symptoms	Follow unresolved abnormalities
Mouth / lungs / infection NCCP 00322 · SMPC	Review mouth, breathing and infection	Mouth-care prevention; report symptoms early	Review symptoms and daily function	Prompt assessment between visits	Confirm recovery and restart conditions

Care / check	Before starting	First eight weeks	Four-week reviews	Longer-term treatment	Hold / change / completion
Bone / endocrine health NCCP 00322 · SMPC	Review fracture risk and existing dual-energy X-ray absorptiometry (bone density) (DXA)	Exemestane symptom support	Follow the agreed bone-health plan	Repeat assessment according to risk	Endocrine and bone plan reviewed separately
Cancer response NCCP 00322 · SMPC	Baseline disease assessment	New symptoms can bring review forward	Imaging according to consultant plan	Continue response and symptom review	Progression prompts a new treatment decision

What each check is for

1 Before the first dose

Before starting

Check eligibility, organ function and the exact treatment/support plan.

Ask / act: Confirm who arranges each check and who reviews it.

2 Early toxicity checks

First eight weeks

Important toxicity can begin before the first routine review.

Ask / act: Keep early appointments and use the agreed symptom-contact plan.

3 Benefit, tolerability and adherence

Longer-term treatment

Longer-term treatment still needs clinical review.

Ask / act: Ask about symptom support, results and the next decision.

4 Completion or treatment change

Hold / change / completion

The targeted medicine and endocrine partner may have different stop dates.

Ask / act: Confirm each medicine and the ongoing follow-up plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Baseline tests and ongoing tests have different purposes.

Before the first dose

full blood count (FBC), renal and liver profile; blood urea nitrogen and urinary protein. Blood glucose, total cholesterol and triglycerides; optimise pre-existing metabolic abnormalities. Review lung symptoms, infection, mouth health, all medicines, planned surgery/radiotherapy and exemestane bone risk.

During treatment

FBC, renal/liver profile, total cholesterol and triglycerides every four weeks or as clinically indicated. Fasting glucose as clinically indicated, with closer surveillance for diabetes or increased risk. At reviews assess mouth ulcers, intake, infection, cough/breathlessness, oedema, skin/bowel symptoms and adherence. Disease assessment and imaging follow the consultant plan; new symptoms can bring review forward.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Treatment, checks and next decisions

No dates or personal information are needed. The team confirms individual appointments and any treatment changes.

Treatment or check	Timing in this view
Start safely	**Before starting** Confirm eligibility, prescription, baseline tests and support plan
Active treatment	**First eight weeks → Four-week reviews → Longer-term treatment** Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.
Toxicity and benefit reviews	**First eight weeks → Four-week reviews → Longer-term treatment → Hold / change / completion** Keep the scheduled tests and report important symptoms between visits
Completion or change	**Hold / change / completion** Continue while beneficial and tolerated; review each medicine separately if everolimus is held.

Take everolimus at the same time daily, consistently with food or consistently without food. Take exemestane after a meal. Swallow tablets whole with water; do not crush or chew. If a dose is missed, take the next scheduled dose without adding an extra dose.

Columns show sequence, not equal lengths of time.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Living with treatment

Supportive medicines and the endocrine partner have their own plan.

Nausea support

Everolimus is oral minimal–low emetic risk. No routine infusion premedication is needed; agree individual rescue treatment and local oral-systemic anticancer therapy (SACT) advice.

Mouth care from the start

Agree non-alcoholic mouth care and whether a prescribed alcohol-free corticosteroid mouthwash is appropriate during the first eight weeks. Report mouth ulcers early and check for thrush rather than adding an antifungal automatically.

Lungs and infection

New cough, breathlessness or fever needs prompt review. If systemic steroids or other immunosuppression are used, the team considers Pneumocystis jirovecii pneumonia (PJP) prevention according to risk.

Metabolic and bone health

Keep glucose, cholesterol, kidney and liver checks. Discuss diabetes/lipid care, exemestane bone risk, exercise, diet and any indicated bone-protection treatment.



Surgery, radiotherapy and other medicines

Tell both oncology and procedure teams about everolimus before surgery or radiotherapy. Check all new medicines and supplements; avoid grapefruit and St John’s wort.

When the plan changes

Continue, hold or stop

The team confirms the plan for each medicine and the next review. Do not restart a clinically held treatment from this educational timeline.

Practical support

Nausea support

Mouth care from the start

Lungs and infection

Metabolic and bone health

Surgery, radiotherapy and other medicines

Taking tablets / attending treatment

Practical treatment details

Supply and appointments

Request the next supply before tablets run out.

This oral treatment does not itself require a central line or infusion pump.

Questions to ask

Which checks happen before and during treatment?

FBC, renal and liver profile; blood urea nitrogen and urinary protein. Blood glucose, total cholesterol and triglycerides; optimise pre-existing metabolic abnormalities. Review lung symptoms, infection, mouth health, all medicines, planned surgery/radiotherapy and exemestane bone risk. FBC, renal/liver profile, total cholesterol and triglycerides every four weeks or as clinically indicated. Fasting glucose as clinically indicated, with closer surveillance for diabetes or increased risk. At reviews assess mouth ulcers, intake, infection, cough/breathlessness, oedema, skin/bowel symptoms and adherence. Disease assessment and imaging follow the consultant plan; new symptoms can bring review forward.

Do I need to enter dates?

No. This map shows general timing without personal information. Your hospital arranges actual appointments; ask who books each test and who reviews its result.

What if a dose is missed or treatment is held?

Take everolimus at the same time daily, consistently with food or consistently without food. Take exemestane after a meal. Swallow tablets whole with water; do not crush or chew. If a dose is missed, take the next scheduled dose without adding an extra dose. A hold may affect one medicine rather than the whole combination; confirm each component.

What happens after treatment changes or ends?

Continue while beneficial and tolerated; review each medicine separately if everolimus is held. Keep the separately agreed disease follow-up, endocrine and bone-health plan.

Explain the plan back: Which medicine is taken when, which test is due next, who reviews it, and which symptoms mean you should call today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Everolimus + exemestane · supportive-care plan

Active medicines: Everolimus + Exemestane

Emetogenic risk: Oral minimal–low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Everolimus is oral minimal–low emetic risk. No routine infusion premedication is needed; agree individual rescue treatment and local oral-systemic anticancer therapy (SACT) advice.

- Agree non-alcoholic mouth care and whether a prescribed alcohol-free corticosteroid mouthwash is appropriate during the first eight weeks. Report mouth ulcers early and check for thrush rather than adding an antifungal automatically.
- New cough, breathlessness or fever needs prompt review. If systemic steroids or other immunosuppression are used, the team considers Pneumocystis jirovecii pneumonia (PJP) prevention according to risk.
- Keep glucose, cholesterol, kidney and liver checks. Discuss diabetes/lipid care, exemestane bone risk, exercise, diet and any indicated bone-protection treatment.
- Tell both oncology and procedure teams about everolimus before surgery or radiotherapy. Check all new medicines and supplements; avoid grapefruit and St John’s wort.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

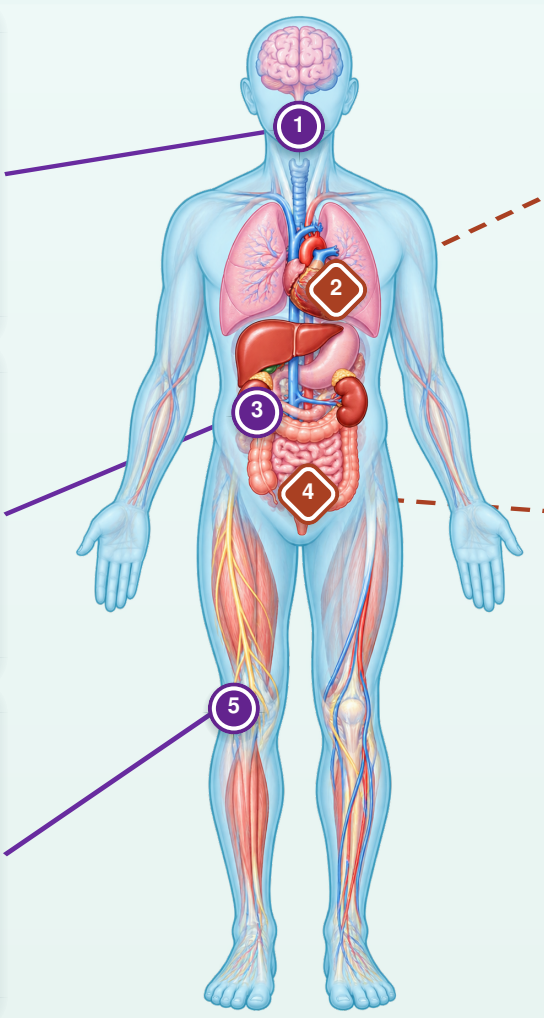
Everolimus + exemestane

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.

Oral targeted therapy + hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Mouth soreness

everolimus

Often develops early; report ulcers before eating or drinking becomes difficult.

2 New cough or breathlessness

everolimus

Prompt assessment is needed for pneumonitis, infection and other causes.

3 Blood glucose and lipids

everolimus

Changes may be silent; keep blood-test appointments and report marked thirst or frequent urination.

4 Diarrhoea or reduced intake

everolimus

Report persistent diarrhoea, inability to drink or dehydration.

5 Joints and bone health

exemestane

Joint symptoms can affect activity; bone loss needs risk-based monitoring.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for new or worsening cough or breathlessness, fever or infection symptoms, mouth sores that prevent drinking/eating, severe diarrhoea or dehydration, or marked thirst and frequent urination. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00322 v4 · official regimen**
   https://healthservice.hse.ie/documents/6720/322_v4_Everolimus_and_Exemestane.pdf
2. **EMA Afinitor SmPC; product information updated 20 November 2025, checked 20 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/afinitor-epar-product-information_en.pdf
3. **HPRA Exemestane product information cited in NCCP 00322**
   https://assets.hpra.ie/products/Human/27567/Licence_PA2315-087-001_27042023100604.pdf
4. **BOLERO-2 final progression-free survival analysis**
   https://pubmed.ncbi.nlm.nih.gov/24158787/
5. **BOLERO-2 final OS, Piccart et al 2014**
   https://pubmed.ncbi.nlm.nih.gov/25231953/
6. **SWISH stomatitis-prevention study**
   https://pubmed.ncbi.nlm.nih.gov/28314691/
7. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
8. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
9. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
10. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf